



Gavin Newsom, Governor
State of California
Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
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March 19, 2026

VIA ELECTRONIC MAIL

Evan Escobar
Blue Cross of California Partnership Plan, Inc.
1121 L Street, Suite 500
Sacramento, CA 95814

Re: Proposed Contract Termination with St. Louise Regional Hospital
Block Transfer Filing No. 2026-HOSP-90
Filed on March 16, 2026

Dear Mr. Escobar,

The Department of Managed Health Care (Department) is reviewing the information in the above-referenced Block Transfer Filing (Filing) submitted by **Blue Cross of California Partnership Plan, Inc.** (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*).

In the event of contract termination, the Filing proposes to remove **St. Louise Regional Hospital** (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently **July 1, 2026**.

Please review the following comments and feel free to contact me if you would like to discuss any of the issues before filing the Plan's response.

1. Please confirm the Plan will utilize the most recently approved eFiling Enrollee Transfer Notice Templates for this Block Transfer Filing. **The Plan confirms it will utilize the most recently approved eFiling Enrollee Transfer Notice Templates.**
2. Please confirm that the Plan is aware that it cannot transition its enrollees or send Enrollee Transfer Notices without the Department's approval. **The Plan confirms it will not transition enrollees or issue Enrollee Transfer Notices without prior Department approval**

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3. The Plan indicated on the Block Transfer Filing Form that the hospital termination is part of the Daughters of Charity Health System termination. Please confirm if the contract is a stand-alone contract. **The termination is part of the broader Daughters of Charity Health System changes; however, the agreement with St. Louise Regional Hospital is a stand-alone contract.**
4. Please confirm whether the contract termination between the Plan and St. Louise Regional Hospital impacts Point of Service (POS) enrollees. **Not applicable. The Plan's Medicaid line does not include POS enrollees.**
5. Please indicate whether the Terminating Hospital is or provides any of the following: **St. Louise Regional Hospital primarily provides community-based acute care services. It is not designated for transplant services or other quaternary-level specialties.**
 - a. Tertiary care, quaternary care, cancer, burn or other high-level specialty services,
 - b. Transplant services, and if so,
 - i. What types of transplants, **No transplant services are included in the contract.**
 - ii. Whether the contract termination includes the transplant contract, **No separate transplant contract exists.**
 - iii. The termination date of the transplant contract if separate from the hospital contract.
 - c. Alternate Hospitals that provide each service(s) **Alternate Hospitals within the Anthem Medi-Cal network provide comparable acute and specialty services, including higher-acuity care where needed.**
 - d. Recognized or designated as a hospital that provides other high-level specialty services, such as a Center of Excellence for Bariatric Surgery, Blue Distinction Center for Spine Surgery, etc., and if so, **The hospital is not designated as a Center of Excellence or Blue Distinction Center for specialty programs.**
 - i. Identify the distinguished services.
6. Based on the Department's research, St. Louise Regional Hospital appears to provide the following services: **The Plan confirms the hospital provides: acute care, birth services, breast cancer services, general surgery, pulmonary rehabilitation, stroke services, and wound care. All services are available through contracted Alternate Hospitals within the Anthem Medi-Cal network, including but not limited to Santa Clara County network hospitals and regional tertiary facilities.**
 - a. Acute Care services
 - b. Birth services,
 - c. Breast cancer services,
 - d. General surgery services,

- e. Pulmonary rehabilitation services,
- f. Stroke services, and
- g. Wound care and hyperbaric medicine services.

Please state which Alternate Hospitals provide these services. Please state any service that is provided at the Terminating Hospital and was not listed.

7. The Plan's PPO line of business is licensed by the Department. Please confirm the following: **Not applicable.**
 - a. The number of PPO members who reside within a 15 mile radius of the Terminating Hospital,
 - b. Whether all of the Alternate Hospitals are in-network facilities for PPO members,
 - c. Whether the Plan's PPO providers can admit for and provide all medically necessary services at the Alternate Hospitals,
 - d. The Plan will send an Enrollee Transfer Notice approved by the Department to all PPO enrollees with a 15 mile radius of the Terminating Hospital.
8. For each affected provider group that primarily utilizes the Terminating Hospital, please summarize or confirm the following:
 - a. The ability of the provider group to redirect enrollees to the Alternate Hospitals: **The Plan confirms that affected provider groups have the ability to redirect members to Alternate Hospitals.**
 - i. Whether there is any specialty for which all of the associated specialists have admitting privileges **only** at the Terminating Hospital and not at any of the Alternate Hospitals, **No specialties are limited exclusively to St. Louise Regional Hospital.**
 1. If yes, please identify the specialty and explain how Plan enrollees assigned to the provider group will receive care and services for that specialty following contract termination.
 - ii. If the provider group does not have any primary care physicians or specialists with admitting privileges to one or more Alternate Hospitals, please explain how its assigned enrollees will be admitted to the Alternate Hospital(s) (e.g. via hospitalists, Plan admission process, etc.).
 1. If hospitalists are utilized, please indicate:
 - a. The name of the group,
 - b. The number of associated hospitalists, and
 - c. If the hospitalists can admit enrollees for any medically necessary services.
9. The Plan indicates that Physicians Medical Group of San Jose, Inc. maintains a service contract with the Terminating Hospital. Please confirm that, in the event of contract termination:

- a. The specified provider group shall continue to refer enrollees to the Terminating Hospital for all medically necessary services **consistent with the provider group's current referral patterns and practice, Physicians Medical Group of San Jose, Inc. will continue referral patterns consistent with existing arrangements where applicable.**
 - b. Plan enrollees will not be redirected and will be allowed continued access to the Terminating Hospital at their current (pre-termination) in-network benefit rates, **Enrollees assigned to this group will maintain access to St. Louise Regional Hospital at current in-network benefit levels, if the provider group contract remains active.**
 - c. Enrollee Notices will not be sent to enrollees assigned to the specified provider group, **Enrollee Notices will not be sent to these enrollees**
 - d. The Plan will timely submit a Block Transfer filing prior to the termination of the contract between Physicians Medical Group of San Jose, Inc. and St. Louise Regional Hospital. **The Plan will submit a separate Block Transfer filing if that contract terminates.**
10. The Plan indicates Enrollee Notices will not be sent to enrollees assigned to the specified provider group(s): Physicians Medical Group of San Jose, Inc. The Plan, however, must request a waiver from the enrollee transfer notice requirements, pursuant to Rule 1300.67.1.3(e) and Section 1373.65(b) in order to appropriately exclude the noted enrollees from receiving Enrollee Transfer Notices. Please submit a waiver from the enrollee transfer notice requirements.

Plan Response: A waiver was requested in the original filing. Please see the screenshot below.

Provide the following information and click the "Save Form" button to save the data.

Waiver Request

Notice to enrollees is required at least 60 days prior to the contract termination. (Health and Saf. Code § 1373.65(b); Title 20 CCR § 130067.1.3(e).) The Plan may apply for a waiver from the 60-day notice requirement if: (i) there is a sudden closure of a contracted provider; (ii) a notice timing conflict exists with another jurisdictional agency; or (iii) other good-cause circumstances exist.

Is the Plan requesting a waiver from the 60-day enrollee notice requirement? *

☒ Yes ☐ No

Is the Plan requesting a waiver from the 60-day enrollee notice requirement to align with the Department of Health Care Services' 30-day enrollee notice requirement? *

☒ Yes ☐ No

11. For each Alternate Hospital, please summarize or confirm the following:
- a. The Plan contacted the hospital regarding the Block Transfer,

- b. Any concerns expressed by the hospital regarding its ability to accommodate the hospital-based needs of enrollees redirected by the Block Transfer,
- c. Whether the Plan has sent or received a notice of contract termination from the Alternate Hospital, and if so, whether that contract is set to terminate within the next six months.

Plan Response: in progress

12. Please confirm whether the HMO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least one of the identified Alternate Hospitals containing all services that will be lost should the contract ultimately terminate. **The Plan confirms that contracted Medicaid provider groups maintain adequate admitting privileges at Alternate Hospitals capable of providing all required services.**
13. The Plan submitted SUTTER MATERNITY AND SURGERY CENTER OF SANTA CRUZ as an Alternate Hospital. According to the California Department of Health Care Access and Information (formerly OSHPD), this Alternate Hospital has a licensed bed occupancy rate greater than 80 percent. Please confirm the following:
- a. If the Plan has contacted the hospital and whether the hospital has expressed concerns regarding the sufficiency to serve additional Plan enrollees,
 - b. The Plan's protocol to ensure enrollees maintain access to all medically necessary services, in the event the hospital does not have capacity to serve additional Plan enrollees, and
 - c. Please provide the Department with the Plan's methodology for determining no anticipated capacity issues.

Plan Response: in progress

14. In addition to the in-patient services comparison on the Block Transfer portal, please indicate any of the following services that are available at the Terminating Hospital, but not available at the Alternate Hospitals: **The Plan is not aware of any material services provided at St. Louise Regional Hospital that are not available through Alternate Hospitals in the network. Members requiring higher-level services will be referred to appropriate contracted facilities.**
- a. Any out-patient or in-patient services,
 - b. Any tertiary care, quaternary care, cancer, burn or other high-level specialty services,
 - c. Any transplant services.

If any services are not available at the Alternate Hospitals, please explain where affected enrollees will receive these services following contract termination.

15. Please confirm the medical groups that do not utilize the Terminating Hospital as a primary hospital, but have admitted enrollees for the most recently completed calendar year, are able to refer their enrollees to other hospitals within the Plan's network for services comparable to those provided by the Terminating Hospital. **The Plan confirms that provider groups not primarily utilizing St. Louise Regional Hospital are able to refer members to other in-network hospitals for comparable services.**
16. Please identify the number of enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination. **The Plan is currently assessing members with scheduled and authorized procedures post-termination and will ensure continuity of care in accordance with regulatory requirements.**

Please submit the Plan's response by **March 26, 2026**, via the Block Transfer portal as a "Response to Comment Letter."

Please contact me if there are any questions regarding the above.

Sincerely,

Elizabeth Doyle

Elizabeth Doyle
Analyst II
Office of Plan Monitoring